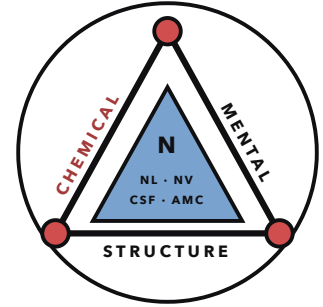


The Chemical Leg



You already know how to find it. This is the other half of the job: **saying why**. A working reference for turning a muscle test finding into a nutritional recommendation you can defend to a patient, a professor, or an MD.

BUILT FOR AK CLUB ATL

Presented by NutriDyn · Asher Allen

COVERS

22 organ & gland reflex points, the muscles and meridians behind them, and the biochemistry underneath each recommendation.

BUILT ON

Goodheart's Triad and Five Factors. Chapman's neurolymphatic reflexes. Rakowski's organ and gland receptor chart.

ANSWERS

Why the system is asking. What in the bottle does the work. How much. How long. And when to stop and refer out.

Your crest has three sides.

Everyone else on campus argues about one of them.

Walk the technique clubs at Life. Gonstead, Thompson, Blair, NUCCA, Torque Release, Pierce. Every one of them is doing serious work on the base of the triangle. Structure. It is the base for a reason, and nobody in this building needs convincing.

Applied Kinesiology is one of the only techniques on this campus that puts all three sides in the same exam. Structural, chemical, mental. It is on your logo. It is the first thing Goodheart taught. And of the three, the **chemical side is the one students can find but not yet explain.**

That gap is real and it is specific. A second-quarter student can learn to find an inhibited popliteus in an afternoon. Explaining, out loud, why that patient's bile flow matters, what bile acids and taurine actually do to a fat globule, and why poor bile flow quietly wrecks the vitamin D you just recommended, is a different skill. Nobody teaches it in a club meeting because it takes time nobody has.

So we wrote it down.

WHAT YOUR PRESIDENT ASKED US FOR

THE ASK, AS IT CAME TO US

"When they are doing muscle testing they need to be able to realize *why* they are testing it. If the gallbladder point tests weak and they reach for Lipo-Flow, they need to know more than that. Why does it work? Why is it needed? What is in the product that helps the biochemistry of this patient? And how much do you give?"

That is a better question than most practicing doctors ask. Every page after this one is built to answer it, in that order, for every reflex point on the chart.

WHAT THIS IS, AND WHAT IT IS NOT

IT IS

- A bridge from a finding on the table to a recommendation you can say out loud.
- Real biochemistry, with the citations attached, so you can check us.
- Dose ranges and a defined endpoint, so you know when it worked and when it did not.
- A referral line on every single system, because you are students and that line matters.
- Honest about where the evidence is thin. Page 15 is the one nobody else would print.

IT IS NOT

- A claim that a muscle test diagnoses disease. ICAK does not claim that, and neither do we.
- A replacement for history, exam, or labs. It sits on top of them.
- A catalog. Products appear only where there is a mechanism to justify them.
- Scope to treat. You are students. Everything here is under supervision.

THE ONE RULE THAT MAKES ALL OF THIS DEFENSIBLE

Never let the muscle test be the only thing holding up your recommendation. ICAK's own 1992 Status Statement says applied kinesiology "is not intended to be used as a single method of diagnosis" and "should enhance standard diagnosis, not replace it." Follow that and you are standing on the governing body of your own technique. Ignore it and you are standing on nothing, and the first person who reads the literature will take your legs out. Page 15 shows you exactly which papers they will bring.

A QUESTION YOUR UNIVERSITY ALREADY ASKS YOU

Life's own recruiting material asks: **"What kind of chiropractor will you be?"** In this room the answer is not "the one who muscle tests." Half this campus muscle tests. The answer worth having is **the one who can explain why.** That is a smaller room, and it is the room patients stay in.

The Four Questions

Every finding on the chart runs the same road. Learn the road once and all 22 points work the same way. If you cannot answer all four out loud, you are not ready to recommend.

0

Test in the clear, then find it.

Baseline first. The indicator muscle grades facilitated with no touch, no substance, no challenge. A muscle that will not test clear is unusable and everything downstream of it is noise. Then work the chart: an inhibited muscle, a tender neurolymphatic point, a positive therapy localization. **Therapy localization tells you where. It does not tell you what.**

1

Does anything else agree with it?

"What else in this patient points the same direction?"

This is the step that separates a doctor from a technician, and it is the step students skip. Before the finding becomes a recommendation it needs a second witness: history, symptom pattern, diet, medications, physical exam, or a lab. The **Confirm With** column on pages 4 and 5 exists only for this. One finding is a hypothesis. Two findings that agree is a clinical picture.

2

Why would that system be asking for help?

"What is this organ actually trying to do, and what is it short on?"

Name the physiology. Not "the gallbladder is weak" but "bile is what emulsifies fat into micelles, and without micelle formation this patient absorbs neither their dietary fat nor the A, D, E and K riding in it." When you can say the mechanism, the patient hears a doctor. When you cannot, they hear a vitamin salesman. So does the MD reading your notes.

3

What is in the bottle that does that work?

"Which specific ingredient touches the mechanism I just named?"

A product recommendation is only as good as the actives inside it. Ox bile supplies the conjugated bile acids directly. Taurine is a conjugation partner. Betaine HCl acidifies the stomach and there is instrumented human data on exactly how far and for how long. If you cannot point at the ingredient that does the job, pick a different product.

4

How much, for how long, and what should change?

"What is the dose, and what is my endpoint?"

Dose and timing are part of the recommendation, not an afterthought. So is the endpoint. Decide before the patient leaves what you expect to change and by when, then reassess it. "Take this and see how you feel" is not a plan. **Two to four weeks with one named marker** is a plan.

AND THE QUESTION UNDERNEATH ALL FOUR

ASK IT EVERY TIME

Is there anything here that should not be treated with nutrition at all? Every system card in this guide carries a red *Stop and refer* line. Learn those before you learn the products. The fastest way to end a career is to hand a supplement to someone who needed a hospital. Right upper quadrant pain with fever and jaundice is not a bile support case. Unexplained weight loss is not an adrenal case.

EVERY CARD IS LABELED BY QUESTION NUMBER

From page 8 on, each system card carries **Q2** (why it is asking), **Q3** (what does the work), **Q4** (how much) and **Q5** (stop and refer) down its left edge. Q1, the second witness, is the **Confirm With** column on pages 4 and 5. Open to any page, read down the red labels, and you know which question you are answering.

A NOTE ON LANGUAGE, IT COSTS YOU CREDIBILITY

Say **inhibited** and **facilitated**, not weak and strong. Say **HPA-axis dysregulation**, not adrenal fatigue. Say **intestinal permeability**, not leaky gut. Say **suggests**, not **diagnoses**. Every swap is free, and every one keeps you in the conversation with the people who can refer to you.

The Foundation Five

Reflex-specific recommendations are the second thing you do. This is the first. Correcting a foundational shortfall sits upstream of nearly every finding on the chart, and a targeted product landing on a depleted patient is a product that underperforms.

ESSENTIAL MULTI Cofactor coverage. Nearly every enzyme on the next ten pages needs a B vitamin, a mineral, or both to run.	D3 5000 WITH K2 D3 5,000 IU with MK-7 60 mcg. Fat soluble, so it depends on the bile chain on page 6.	OMEGA PURE EPA-DHA 720 EPA 860 mg and DHA 580 mg per 2 softgels. Membrane phospholipid and eicosanoid substrate. Also bile dependent.	ULTRABIOTIC DAILY Roughly 30 billion CFU across 7 strains, into a barrier that turns over every three to five days.	FRUITS & GREENS Polyphenol, fiber and nitrate density most patients will not reach from food this week.
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WHY FOUNDATION FIRST IS A CLINICAL ARGUMENT, NOT A SALES ARGUMENT

Because the targeted product may not get absorbed. Three of the five above are fat soluble or fat dependent. If the patient's bile flow is poor, which is exactly what a popliteus finding is asking you about, the D3 and the omega-3 are being swallowed and not absorbed. Fix the chain, not the link.	Because it is the same advice for everyone, which makes it easy to follow. Compliance is the silent variable in every protocol you will ever write. A patient who is taking five things reliably beats a patient who was handed nine and takes three.
Because deficiency is common and boring and real. It rarely presents as one dramatic finding. It presents as several systems testing mediocre at once. When three or four unrelated points are all tender, suspect the substrate before you suspect four separate organs.	Because it gives you somewhere honest to stand. The evidence for correcting a documented deficiency is not controversial. Nobody is going to argue with you about whether a deficient patient should be repleted. Start every case on ground you can hold.

WHAT THE LITERATURE DOES AND DOES NOT SUPPORT HERE

Be precise, because students who overclaim on foundation nutrients get corrected in public. What follows is the defensible version.

NUTRIENT	CLAIM YOU CAN MAKE AND DEFEND	CLAIM TO AVOID, AND WHY
Vitamin D3	Correcting documented deficiency. Bone and calcium homeostasis. Immune signaling.	Cancer or cardiovascular prevention in replete people. VITAL randomized 25,871 adults to 2000 IU daily for a median 5.3 years and found no reduction in invasive cancer or major cardiovascular events. Know this before someone quotes it at you.
Vitamin K2	MK-7 at 180 mcg daily for three years in 244 healthy postmenopausal women improved arterial stiffness and cut dephosphorylated uncarboxylated MGP by half.	Vague "K2 directs calcium to bone" language. You have a real trial, so use it. And note the trial dose was 180 mcg while D3 5000 with K2 supplies 60 mcg. Knowing when your product sits below the study dose is exactly the kind of precision that earns trust.
Omega-3	Biomarkers. Roughly 15 percent dose-dependent triglyceride reduction on high-quality evidence, plus modest blood pressure and CRP effects.	Mortality and cardiovascular event reduction. Cochrane pooled 79 trials and 112,059 people and found little or no effect. Claim the triglycerides, not the survival.
Probiotic	IBS symptoms. Pooled across 43 randomized trials, the relative risk of symptoms persisting was 0.79 against placebo.	Strain-specific promises. The same review states plainly that which species and strains are most beneficial remains unclear.
Polyphenols	Cocoa flavan-3-ols across 42 trials improved flow-mediated dilatation and insulin resistance. Dietary nitrate lowers blood pressure, more so under 65.	Blanket antioxidant claims. Effects are compound-specific, dose-specific and often age-specific.

SAY IT THIS WAY AT THE TABLE

"Before we chase anything specific, I want the basics covered, because a few of the things I would reach for next are fat soluble and they need your digestion working to get in. Give me four weeks on the foundation and we will retest. If your picture changes on its own, that told us something too."

Point, muscle, meridian, and what to confirm

Reflex points follow the organ and gland receptor numbering on the Rakowski neurolymphatic chart. The **Confirm With** column is the one that matters. It is step 1 of the Four Questions and it is what turns a finding into a case.

#	SYSTEM	INDICATOR MUSCLE	MERIDIAN	CONFIRM WITH BEFORE YOU RECOMMEND	NUTRIDYN
0	Pituitary / Hypothalamus	Pectoralis major pars abdominalis; supraspinatus	—	Cycle history, thermoregulation, sleep-wake pattern, libido. Endocrine axis complaints belong to a physician first.	Prolan-H, Gynplex, Elo-Plex
1	Parathyroid	Levator scapulae △	Stomach	Cramping, dental history, fracture history. Serum calcium if there is any real suspicion.	Para Thyrolate, HCl Support, Bone Support
2	Paranasal sinuses	Sternocleidomastoid; scaleni; splenius	Stomach	Seasonality, dairy and food timing, post-nasal drainage, prior antibiotic courses.	Aller Pro, NutraZyme, UltraBiotic Daily Multi-Strain
3	Thyroid	Teres minor	Triple Warmer	Labs, not optional. TSH, free T4, TPO antibodies. Cold intolerance, hair, bowel transit, cycle.	Thyroid Complex, Thyroid Pro, Organo Iodine Drops
3	Heart	Subscapularis; subclavius	Heart	Exertional tolerance, blood pressure, statin use, family history. Chest pain is a referral, never a protocol.	Cardio Fit, CoQ10 200 mg, Dynamic Cardio Flow
4	Pineal	Teres major △	Governing Vessel	Sleep onset versus sleep maintenance, screen exposure, shift work, travel.	Pineal Concentrate, Melatonin 3
5	Brain	Supraspinatus	Central Vessel	Concussion history, cognitive complaint pattern, sleep, omega-3 intake, alcohol.	Brain Restore, Brain Support, Dynamic Inflam-Eze
6	Lung	Deltoid; serratus anterior; coracobrachialis	Lung	Smoking and vaping, occupational exposure, exercise tolerance, seasonal pattern.	Lung Support, NAC-600, Spectrum BR
7	Thymus	Infraspinatus	Triple Warmer	Infection frequency and duration, wound healing, vitamin D status, stress load.	Thymtrate, Coryza Forte, Immune Support
8	Liver	Pectoralis major pars sternocostalis (phase I); rhomboideus (phase II)	Liver	Alcohol, medication list, occupational solvents, body composition. LFTs and GGT if suspicion is real.	LiverCare, Dynamic Detox, Detox Support, Livatrate
9	Gallbladder	Popliteus	Gall Bladder	Worked in full on page 6. Fat intolerance, stool character, right infrascapular referral, prior cholecystectomy.	Lipo-Flow, Lipo-Complex, Lipo-Plex
10	Eye / Ear	Trapezius pars descendens	Kidney	Screen hours, night vision, tinnitus, dizziness. Sudden change in either is a same-week referral.	Eye Pro, Optex
11	Stomach	Pectoralis major pars clavicularis; biceps brachii	Stomach	Fullness after small meals, reflux, protein tolerance, current PPI or H2 blocker use , iron and B12 status.	HCl Support, Zinc-Carnosine, Dynamic GI Integrity

TWO ENTRIES CARRY A WARNING MARK, AND HERE IS WHY

△ **Teres major and levator scapulae.** Several charts in circulation pair teres major with the pineal and levator scapulae with the parathyroid. In the canonical correspondence tables, teres major belongs to the **Governing Vessel and the spine**, and levator scapulae sits on the **Stomach** meridian. We have printed both readings rather than quietly picking one. If someone at a seminar challenges you on it, the right answer is that the chart lineage is uneven at these two points, and here is the source for each. That answer wins the room. Guessing does not.

Points 12 through 21

#	SYSTEM	INDICATOR MUSCLE	MERIDIAN	CONFIRM WITH BEFORE YOU RECOMMEND	NUTRIDYN
12	Pancreas digestive enzymes	Latissimus dorsi	Spleen	Undigested food in stool, floating or greasy stool, bloating 30 to 60 minutes after eating, weight loss.	Digestive Complete, Lipo-Complex
12	Pancreas hormones	Triceps brachii	Spleen	Fasting glucose, HbA1c, waist circumference, post-meal energy crash, family history.	Glucos IR, Berberine Pro, Dynamic Cardio-Metabolic
12	Spleen	Trapezius pars transversa and ascendens	Spleen	Infection recovery time, iron status, fatigue pattern.	Splenotrate, Immune Support, Coryza Forte
13	Small intestine	Quadriceps femoris (rectus femoris)	Small Intestine	Food reactivity pattern, NSAID and antibiotic history, stool frequency, skin and joint complaints.	Dynamic GI Integrity, Dynamic GI Restore, Dynamic GI Defend
14	Appendix / caecum	Quadratus lumborum Δ	—	Rule out the surgical abdomen first. Right lower quadrant pain with guarding, rebound or fever is an emergency department, not a protocol.	Dynamic GI Restore, Dynamic GI Defend, Spectrum BR
15	Adrenal cortex	Gracilis; soleus	Triple Warmer	Sleep timing, energy curve across the day, caffeine load, stressors, lying-to-standing blood pressure.	Stress Essentials Adrenal Renew, Adrenal B1B6, Cortisol Pro
15	Adrenal medulla	Sartorius; gastrocnemius; tibialis posterior	Triple Warmer	Panic or startle pattern, tremor, palpitations, orthostatic symptoms.	Stress Essentials Balance, Adrenal Resilience, L-Tyrosine
16	Kidney	Psoas major; iliacus	Kidney	eGFR and creatinine before supplementing anything. Blood pressure, hydration, NSAID use, edema.	Renal Complex, Renatrate, Renex
17	Ileocecal valve	Iliacus (flexor); tensor fasciae latae (rotator)	—	Alternating constipation and loose stool, right lower quadrant tenderness, positional bloating.	Dynamic GI Restore, Dynamic Fiber, Muconell
18	Uterus / Prostate	Gluteus medius pars posterior; gluteus maximus	Circulation-Sex	Cycle detail, hormone therapy, urinary stream in men, PSA history where relevant.	Dynamic Hormone Balance, Gynplex, Prostate Support
18	Ovary / Testicle	Piriformis; gluteus medius pars anterior	Circulation-Sex	Cycle regularity, acne and hair pattern, libido, fertility, body composition change.	Estro Balance, Androzyme, Testro Balance, Dynamic Detox
19	Urinary bladder	Fibularis longus and tertius; tibialis anterior; erector spinae	Bladder	Frequency, urgency, dysuria, recurrence count. Fever or flank pain means pyelonephritis until proven otherwise.	Urinary Cleanse, UT Soothe, U-Tract Complex
20	Large intestine	Tensor fasciae latae; biceps femoris	Large Intestine	Bristol stool type and frequency, fiber intake, antibiotic history, bloating timing.	Dynamic GI Restore, Spore Probio, Dynamic Fiber
21	Duodenum	Rectus abdominis Δ ; obliquus abdominis	Small Intestine	Epigastric pain relative to meals, NSAID use, <i>H. pylori</i> history, appetite.	Muconell, Zinc-Carnosine, Digestive Complete

WHERE THESE ENTRIES COME FROM

Δ marks a pairing we could not trace to Goodheart, Walther or ICAK directly. Quadratus lumborum with the appendix, and rectus abdominis with the duodenum, circulate widely in practitioner charts. The canonical table lists rectus abdominis under **small intestine** broadly.

HOW TO HOLD THIS CHART

Ileocecal valve syndrome, by contrast, is genuinely Goodheart's, from 1967. The clinical link is real, the chart entry is downstream, and you should be able to say which is which. Treat the whole thing as a **heuristic map with a documented lineage**, not verified anatomy.

Point 9. Popliteus inhibited.

Now say why.

This is the whole method run end to end on a single case. Every other system card in this guide is the compressed version of this page. Learn this one properly and the rest are pattern matching.

STEP 0 · THE FINDING

Popliteus tests inhibited on the right. The neurolymphatic point at the right costal margin is sharply tender to a pressure the patient did not expect to hurt. Therapy localization is positive.

STEP 1 · WHAT AGREES WITH IT

You ask, and she tells you fried food has bothered her for two years, she gets a dull ache under the right shoulder blade after big meals, and her stool floats and is pale more often than not. She started a vitamin D supplement eight months ago and her last level barely moved.

Now you have a case. Four independent signals agree. Without them you had one muscle test, which is a hypothesis and nothing more.

STEP 0.5 · BEFORE ANYTHING ELSE

Right upper quadrant pain that is severe, colicky, radiating to the right shoulder, or paired with **fever, jaundice, vomiting or a positive Murphy sign** is not a bile support case. That is possible cholecystitis or obstruction and it needs imaging today. Ask before you recommend. This takes eleven seconds and it is the most important eleven seconds on this page.

THE DETAIL THAT MAKES YOU SOUND LIKE A DOCTOR

Her vitamin D level "barely moved" on supplementation. That is not a dosing failure. It is an **absorption** failure, and it is the single best evidence you have that the bile finding is real. Lead with it.

STEP 2 · WHY THE SYSTEM IS ASKING FOR HELP

Bile acids are synthesized from cholesterol in the liver, conjugated to glycine or taurine, and concentrated in the gallbladder between meals. When fat reaches the duodenum, cholecystokinin triggers release. The bile acids are amphipathic, meaning one end is water-loving and one end is fat-loving, so they surround fat droplets and break them into **mixed micelles**.

Here is the part that matters clinically. Micelle formation is not an efficiency improvement. It is a **requirement**. Pancreatic lipase works at the surface of a fat droplet, so emulsification multiplies the available surface area by orders of magnitude. And the products of digestion, along with cholesterol and **vitamins A, D, E and K**, cannot cross the unstirred water layer at the enterocyte brush border without a micelle to carry them.

THE SENTENCE THAT CLOSES THE CASE

"Every fat soluble thing you swallow, including the vitamin D you have been taking for eight months, has to ride into your body inside a bile micelle. If bile flow is poor, you can take the right dose of the right vitamin forever and never absorb it. That is why your level did not move."

Bile acids are also signaling molecules, acting on the nuclear receptor **FXR** and the membrane receptor **TGR5**, where they help regulate hepatic lipid and glucose metabolism and their own synthesis. So "the gallbladder" is not a storage bag. It is a node in lipid and glucose signaling, which is why bile complaints so often travel with metabolic ones.

Step 3. Open the panel.

Name what does the work.

Here is where most people stop thinking. They hear "gallbladder" and reach for the bile product. But there are **two entirely different problems** hiding under that word, they need different formulas, and choosing wrong is why a correct-looking recommendation does nothing.

PROBLEM A · SHE MAKES BILE, FLOW IS SLUGGISH

Help the liver produce and move it. This is a **choleretic** approach. **Lipo-Flow** is built for exactly this, and it contains **no ox bile at all**.

PROBLEM B · BILE ITSELF IS INADEQUATE OR ABSENT

Supply it directly. Post-cholecystectomy, or genuine insufficiency. That needs **ox bile**, which means **Digestive Complete** (350 mg ox bile) or **Lipo-Plex**.

She still has her gallbladder and the complaint is functional, so she is Problem A. Now open the panel.

IN LIPO-FLOW, PER 2 TABLETS	WHAT IT DOES MECHANICALLY	WHY IT FITS HER
Artichoke 400 mg Dandelion 250 mg Gentian 100 mg	Classical choleretics and bitters, held to increase bile production and stimulate flow. Gentian is a bitter acting on the gustatory reflex arc.	Artichoke is the largest single active in the formula. Be straight about the evidence tier though: this is traditional use plus mechanistic reasoning, not randomized trial data. Saying that out loud costs you nothing and buys you enormous credibility.
Taurine 100 mg	One of the two amino acids bile acids are conjugated to. Taurine conjugates stay ionized across a wider pH range than glycine conjugates, so they stay functional further down the intestine.	Supports the conjugation step rather than bypassing it. This is the ingredient that makes it a bile formula rather than a generic liver formula.
L-methionine 100 mg Inositol 150 mg	Classic lipotropics. Methionine is a methyl donor feeding phosphatidylcholine synthesis through the PEMT pathway.	Note what is NOT here: Lipo-Flow supplies no preformed choline at all. It reaches phosphatidylcholine only by the methylation route. A patient who needs choline directly needs Lipo-Plex (124 mg), not this.
Beet 200 mg Tangerine peel 100 mg	Beet supplies betaine, another methyl donor in the same pathway. Citrus peel is traditionally carminative.	Supporting cast. Do not build your explanation on these.

THE HABIT THAT WILL SEPARATE YOU FROM MOST PRACTICING DOCTORS

Read the panel. Do not trust the category name. A product called Lipo-Flow sounds like it contains bile. It does not, and it is a better choice than one that does for this specific patient. Products in the same category are not interchangeable. Before you recommend, find the ingredient that touches the mechanism you just named out loud. If it is absent, or present at a token amount, pick a different product. Most practitioners never build this habit. Build it now while you have time to.

STEP 4 · HOW MUCH, HOW LONG, WHAT CHANGES

Timing beats dose. Bile support is taken **with the fat-containing meal**, not on an empty stomach and not at bedtime. A correct dose at the wrong time does nothing.

Dose: the Lipo-Flow label reads two tablets with a meal three times daily. In practice, start with the largest fat-containing meal and build from there. Always confirm against the current label, since formulas get revised.

Endpoint, named in advance: stool character and post-fat bloating at **two to four weeks**. Retest popliteus. And because this patient handed you a perfect objective marker, **recheck the vitamin D level at three months**. If it moves this time, you did not just help her digestion, you proved your reasoning.

STOP AND REFER

Severe or colicky right upper quadrant pain. Fever. Jaundice or scleral icterus. Clay-colored stool with dark urine. Positive Murphy sign. Unintentional weight loss. Any of these means imaging and a physician, today, and nothing from a bottle.

Post-cholecystectomy patients are a different conversation. There is no gallbladder to support. Bile still flows from the liver but continuously rather than in a meal-timed bolus, which changes what you are actually trying to do.

Why the system is asking

The compressed version of page 6, for every remaining system. Same four questions, same order.

11 Stomach	Pectoralis major clavicularis • biceps brachii • Stomach meridian
Q2 WHY IT IS ASKING	Gastric acid does four jobs, and each one fails visibly. It converts pepsinogen to pepsin so protein digestion can start. It liberates protein-bound B12 so intrinsic factor can bind it. It solubilizes non-heme iron, zinc, calcium and magnesium into absorbable form. And it is a barrier against ingested organisms. Low acid therefore reads downstream as poor protein tolerance, low B12 and iron, and a raised risk of bacterial overgrowth. Note that low acid and high acid can present with identical upper reflux symptoms, which is exactly why the history matters more than the complaint.
Q3 WHAT DOES THE WORK	HCl Support supplies betaine HCl 1,340 mg plus pepsin 90 mg per two tablets . Note how close that betaine dose sits to the study dose, which is unusual and worth knowing: in volunteers made hypochlorhydric with a proton pump inhibitor, 1500 mg of betaine HCl dropped gastric pH from 5.2 to 0.6, with a median time to pH below 3 of 6.3 minutes and an effect lasting about 73 minutes. <i>Yago 2013</i> . Be precise about what that proves. It proves acidification. It does not prove treatment of any condition. The pepsin matters too, since acid without pepsin only does half the job. Zinc-Carnosine supplies zinc-carnosine 17 mg for mucosal integrity.
Q4 HOW MUCH	With protein-containing meals, never on an empty stomach. Titrate from the low end. A warmth sensation means back off one capsule.
Q5 • STOP AND REFER • Never give betaine HCl alongside an active ulcer, current NSAID use, or a PPI or H2 blocker. Dysphagia, unintentional weight loss, vomiting blood, black tarry stool, or new upper GI symptoms over age 50 are endoscopy referrals, not supplement cases.	
12 Pancreas • digestive enzymes	Latissimus dorsi • Spleen meridian
Q2 WHY IT IS ASKING	The exocrine pancreas supplies lipase, protease and amylase into the duodenum against a bicarbonate buffer that raises luminal pH into the range those enzymes need. Insufficiency shows up as fat in the stool, which is why patients describe stool that floats, looks greasy or is hard to flush, along with bloating that begins 30 to 60 minutes after eating rather than immediately.
Q3 WHAT DOES THE WORK	Digestive Complete carries protease 35,000 USP, amylase 35,000 USP, lipase 7,000 USP, plus betaine HCl 600 mg and ox bile 350 mg , so it covers the enzyme and the bile problem at once. Lipo-Complex is the enzyme plus bile salt alternative. Enzyme replacement is standard of care for <i>documented</i> exocrine pancreatic insufficiency. Know the limit as well as the use: in advanced pancreatic cancer, a meta-analysis of four randomized trials covering 194 patients found no significant effect on survival, weight or quality of life. <i>Ammar 2021</i> . Enzymes are for defined insufficiency, not a universal digestive aid.
Q4 HOW MUCH	With the first bite of the meal, not after. Scale to meal size and fat content.
Q5 • STOP AND REFER • Steatorrhea with weight loss, new-onset diabetes in an older adult, or epigastric pain radiating straight through to the back warrants imaging and a physician.	

Barrier, fermentation, fuel

13 Small intestine	Rectus femoris • Small Intestine meridian
Q2 WHY IT IS ASKING	This is where absorption actually happens and where the barrier lives. Enterocytes turn over every three to five days, which is a demanding rebuild and makes this tissue unusually sensitive to substrate supply. Tight junction proteins regulate what crosses between cells. Secretory IgA coats the surface. Glutamine is the preferred fuel of the enterocyte, which is the mechanistic reason it appears in nearly every GI formula you will ever see.
Q3 WHAT DOES THE WORK	Dynamic GI Integrity supplies L-glutamine 3 g and zinc-carnosine 16.1 mg per scoop, alongside DGL, aloe, slippery elm and mucin. Trace each of those to the mechanism above rather than reading the label as a list. Dynamic GI Restore is the larger rebuild, adding MCTs, arabinogalactans and a full amino acid profile. Dynamic GI Defend is a single active, bovine immunoglobulin at 2.5 g, so it does a narrower and different job. Use the term intestinal permeability, which is a measurable construct, rather than "leaky gut," which will get you dismissed by exactly the people you want referring to you.
Q4 HOW MUCH	Powders away from meals so they are not competing for absorption. Give this tissue four to eight weeks. Turnover is fast but repair is not instant.
Q5 • STOP AND REFER • Blood in stool, nocturnal diarrhea that wakes the patient, fever, or unintentional weight loss. Those point at inflammatory bowel disease or worse and need a gastroenterologist. Screen for celiac before removing gluten, because removing it first makes the test unreliable.	
20 Large intestine	Tensor fasciae latae • biceps femoris • Large Intestine meridian
Q2 WHY IT IS ASKING	Water and electrolyte recovery, and fermentation. Colonic bacteria ferment fermentable fiber into short chain fatty acids, and butyrate is the primary fuel of the colonocyte itself. That is a genuinely elegant arrangement: the tissue feeds the bacteria that feed the tissue. Low fiber intake starves that loop.
Q3 WHAT DOES THE WORK	Dynamic Fiber supplies the fermentable substrate. Spore Probio and UltraBiotic Daily Multi-Strain supply organisms. Across 43 randomized trials the relative risk of IBS symptoms persisting on probiotics versus placebo was 0.79. <i>Ford 2014</i>. Quote the caveat too: which species and strains are most beneficial remains unclear.
Q4 HOW MUCH	Increase fiber slowly with water. Going straight to a full dose produces exactly the bloating the patient came in for and ends compliance in three days.
Q5 • STOP AND REFER • Rectal bleeding, a persistent change in bowel habit over age 45, unintentional weight loss, or iron deficiency anemia in a man or a postmenopausal woman. Colonoscopy, not fiber.	
17 Ileocecal valve	Iliacus • tensor fasciae latae • Goodheart, 1967
Q2 WHY IT IS ASKING	Worth knowing because it is genuinely Goodheart's own contribution, documented in 1967, rather than a downstream chart entry. The valve is the one-way gate between the ileum and the cecum. It holds chyme long enough for absorption to finish and keeps colonic bacteria from tracking backward into the small intestine. A valve that does not close properly is the mechanistic story behind bacterial migration upstream. A valve that stays shut backs things up. That is why patients describe the confusing pattern of alternating constipation and loose stool rather than one or the other.
Q3 WHAT DOES THE WORK	Dynamic GI Restore for the rebuild, Dynamic Fiber for transit and bulk, Muconell for mucosal soothing. Address diet alongside it. Roughage and irritants are the classic aggravators taught with this syndrome.
Q4 HOW MUCH	Judge at two to four weeks on the alternating pattern specifically, since that is the complaint most likely to resolve first.
Q5 • STOP AND REFER • Right lower quadrant pain with guarding, rebound tenderness or fever is appendicitis until proven otherwise. That is an emergency department, immediately. Do not palpate around a suspected surgical abdomen hunting for a reflex point.	

The two systems

students overclaim most

15 Adrenal cortex and medulla

Gracilis, soleus · sartorius, gastrocnemius · Triple Warmer

Q0 SAY IT RIGHT

Say HPA-axis dysregulation. Never say adrenal fatigue. This is not us being fussy. "Adrenal fatigue" has no endocrine standing, and the moment you use it in front of a faculty member, an endocrinologist or a well-read patient, every accurate thing you say afterward gets discounted. The underlying observation is real. The label is what costs you.

Q2 WHY IT IS ASKING

The hypothalamic-pituitary-adrenal axis is a feedback loop with a daily rhythm: CRH to ACTH to cortisol, with cortisol feeding back to suppress its own drivers. Under sustained load the *pattern* shifts, most visibly in the morning rise. That is the defensible framing. It is a regulation and rhythm problem, not an organ running out of fuel.

Q3 WHAT DOES THE WORK

Stress Essentials Adrenal Renew, Adrenal B1B6, Cortisol Pro. Adaptogens have real trial data. *Withania somnifera* at 240 mg daily for 60 days significantly reduced morning cortisol and anxiety scores against placebo *Lopresti 2019*, and 300 mg twice daily for 8 weeks improved perceived stress and serum cortisol *Choudhary 2016*. *Rhodiola rosea* SHR-5 at 576 mg daily for 28 days improved burnout and attention measures in stress-related fatigue *Olsson 2008*. B vitamins are cofactors in steroidogenesis and catecholamine synthesis, which is the honest mechanism for the B-complex component.

Q4 HOW MUCH

Adaptogens in the morning and early afternoon. Evening dosing disrupts the sleep you are trying to fix. Judge at **four to eight weeks** on a named marker: time to fall asleep, energy at 3pm, or a validated stress scale.

Q5 • STOP AND REFER • True adrenal insufficiency is a medical emergency in crisis and it does not look like tiredness.

Hyperpigmentation, orthostatic hypotension, salt craving, hyponatremia, unexplained weight loss, persistent vomiting. That patient needs ACTH stimulation testing and a physician now. Also refer anyone on corticosteroids before adding anything.

3 Thyroid

Teres minor · Triple Warmer meridian

Q2 WHY IT IS ASKING

This one has beautiful, teachable biochemistry. **Tyrosine** residues on thyroglobulin are iodinated by thyroid peroxidase using **iodine** and hydrogen peroxide, and coupled into T4 and T3. Two things follow directly. First, that reaction generates peroxide inside the gland, so the gland needs **selenium**-dependent glutathione peroxidase to protect itself. Second, the deiodinases that convert T4 into active T3 in peripheral tissue are *also* selenoenzymes. Selenium therefore sits on both sides of the reaction, protection and activation. That is why it keeps appearing, and now you can explain it instead of just repeating it.

Q3 WHAT DOES THE WORK

Thyroid Complex, Thyroid Pro, Organo Iodine Drops. On selenium, in autoimmune thyroiditis, TPO antibodies fell significantly at three months *Wichman 2016, Toulis 2010*. Give the caveat the meta-analysts give: an overview of systematic reviews rated the certainty of that evidence low to very low, and whether antibody reduction produces clinical benefit remains to be demonstrated *Wang 2023*. On tyrosine, the biochemistry is textbook, but there is no good trial evidence that supplementing it improves thyroid function in euthyroid people. Do not imply otherwise.

Q4 HOW MUCH

Thyroid tissue responds slowly. Nothing here is judged before **three months**, and the judgment is made on labs, not on how the patient feels this week.

Q5 • STOP AND REFER • A muscle test never replaces TSH, free T4 and TPO antibodies. Overt hypothyroidism is treated with levothyroxine, and delaying that with supplements is real harm. **Iodine can worsen autoimmune thyroiditis**, so establish antibody status before giving it. Any palpable nodule, rapid gland enlargement, hoarseness or difficulty swallowing needs imaging. Patients already on thyroid medication need physician coordination, not a parallel protocol.

The rest of the chart

8 Liver	Pectoralis major sternocostalis · rhomboideus · Liver meridian
Q2 WHY IT IS ASKING	Biotransformation runs in phases and the sequence is the whole clinical point. Phase I , largely cytochrome P450, adds or exposes a reactive group. That intermediate is frequently <i>more</i> reactive than the parent compound. Phase II conjugates it to something water soluble by glucuronidation, sulfation, glutathione conjugation, methylation, acetylation or amino acid conjugation. Phase III exports it. The clinically useful consequence: driving Phase I without Phase II capacity raises the reactive intermediate load . That is why a well-built detox formula pairs inducers with conjugation substrates instead of stacking inducers, and it is a point most practitioners cannot articulate.
Q3 WHAT DOES THE WORK	Dynamic Detox, LiverCare, Detox Support. The chart pairs pectoralis major sternocostalis with phase I and rhomboideus with phase II, which is a genuinely useful distinction if you test both. Glycine, taurine, cysteine and sulfur substrates feed conjugation. Note that bile is the export route for many conjugates, which ties this card directly back to page 6.
Q4 HOW MUCH	Ensure bowels are moving before starting anything that mobilizes. Mobilizing without an exit route is how patients feel worse on a detox protocol.
Q5 • STOP AND REFER • Jaundice, ascites, easy bruising, confusion, or a history of hepatitis or heavy alcohol use. Get liver function tests before recommending anything hepatically active. Herb and drug interactions through CYP450 are real, so read the medication list.	
3 Heart	Subscapularis · subclavius · Heart meridian
Q2 WHY IT IS ASKING	Cardiac muscle has the highest mitochondrial density of any tissue in the body because it never rests. Coenzyme Q10 carries electrons between complexes I and II and complex III in the electron transport chain, so it is directly on the ATP path. The interaction worth knowing cold: CoQ10 is made through the mevalonate pathway , the same pathway statins inhibit at HMG-CoA reductase. Statins lower CoQ10 as a direct mechanistic consequence, not a coincidence.
Q3 WHAT DOES THE WORK	CoQ10 200 mg, Cardio Fit, Dynamic Cardio Flow. Magnesium is worth naming separately: across 34 double-blind placebo-controlled trials in 2,028 people, a median 368 mg daily for a median 3 months lowered systolic pressure by 2.00 mmHg and diastolic by 1.78 mmHg. <i>Zhang 2016</i> . Modest, real, and citable.
Q4 HOW MUCH	CoQ10 is fat soluble, so take it with a fat-containing meal. Which, again, means it depends on the bile chain on page 6.
Q5 • STOP AND REFER • Chest pain is never a supplement conversation. Exertional chest pressure, pain radiating to the jaw or left arm, syncope, new or worsening shortness of breath, or palpitations with lightheadedness go to emergency care. CoQ10 and vitamin K2 both interact with warfarin.	
12 Pancreas · hormones	Triceps brachii · Spleen meridian
Q2 WHY IT IS ASKING	Insulin resistance is a signaling problem before it is a glucose problem, which is why fasting glucose can look acceptable for years while fasting insulin and waist circumference tell the real story. Berberine activates AMP-activated protein kinase, the cell's low-energy sensor, which increases glucose uptake and reduces hepatic glucose output. Chromium participates in insulin signaling.
Q3 WHAT DOES THE WORK	GlucO IR, Berberine Pro, Dynamic Cardio-Metabolic. Diet and resistance training outperform every supplement in this category, and saying so out loud is what makes the supplement recommendation credible.
Q4 HOW MUCH	Berberine is typically divided across meals due to short half-life and GI tolerance. Judge on HbA1c at three months , since that reflects roughly 90 days of glycemic history.
Q5 • STOP AND REFER • Anyone on glucose-lowering medication needs physician coordination, because additive effects can cause hypoglycemia. Berberine interacts with a long list of drugs through CYP450 and P-glycoprotein. Check the medication list first. Polyuria, polydipsia and weight loss need same-week workup.	

Neural, immune, respiratory, renal

5 Brain Supraspinatus Q2 WHY IT IS ASKING DHA is a structural phospholipid of neuronal membranes and is concentrated at the synapse, where membrane fluidity affects receptor function. The brain is also roughly 60 percent lipid by dry weight, which is why omega-3 status is a structural question here and not just an anti-inflammatory one. Q3 WHAT DOES THE WORK Brain Restore, Brain Support, Dynamic Inflamm-Eze. Sleep and exercise outperform all of it and should be said first. Q5 • REFER • Sudden cognitive change, focal neurological deficit, or new severe headache. Image, do not supplement.	7 Thymus / immune Infraspinatus Q2 WHY IT IS ASKING The thymus is where T cells mature, and it involutes steadily with age. Vitamin D receptors are expressed on most immune cell lines and the vitamin modulates both innate and adaptive responses. Zinc is required for lymphocyte development and function. Q3 WHAT DOES THE WORK Immune Support, Thymtrate, Coryza Forte, plus foundation vitamin D. Q5 • REFER • Recurrent serious or unusual infection, persistent lymphadenopathy, night sweats with weight loss. Work that up.
6 Lung Deltoid • serratus anterior • Lung meridian Q2 WHY IT IS ASKING Gas exchange happens across an epithelium under constant oxidative load, which is why the airway lining maintains a substantial glutathione pool. N-acetylcysteine supplies cysteine, the rate-limiting amino acid in glutathione synthesis, and is also a direct mucolytic through disulfide bond reduction in mucus glycoproteins. That is two distinct mechanisms in one molecule, which is unusual and worth being able to name. Q3 WHAT DOES THE WORK NAC-600 supplies 600 mg N-acetylcysteine per capsule. Lung Support and Spectrum BR for the broader picture. Vitamin D belongs in the conversation here too, given its role in respiratory immune signaling. Q5 • STOP AND REFER • Hemoptysis, unexplained persistent cough over three weeks, night sweats with weight loss, or any resting shortness of breath. Image and refer.	
16 Kidney Psoas major • iliacus • Kidney meridian Q2 WHY IT IS ASKING Filtration, electrolyte and acid-base balance, blood pressure regulation through renin, erythropoietin, and the final hydroxylation that activates vitamin D. That last one is worth knowing: significant renal impairment impairs vitamin D activation, which changes how you think about a foundation recommendation. Q3 WHAT DOES THE WORK This is the system where supplements most readily cause harm rather than merely fail to help. Reduced filtration alters handling of magnesium and potassium in particular. Get eGFR and creatinine before you recommend anything. Renal Complex, Renatrate only with known function and, ideally, physician awareness. Q5 • STOP AND REFER • Reduced eGFR, hematuria, new edema, uncontrolled hypertension, or flank pain with fever. When in doubt on this system, recommend nothing and refer. That is the correct clinical answer and nobody will ever fault you for it.	

When to take it, and when to judge it

Timing is a clinical decision, not a convenience. Amounts come off the current label. The timing logic and the endpoint below are yours to own.

CATEGORY	WHEN	JUDGE AT	THE MARKER YOU NAME IN ADVANCE
Bile support	With the fat-containing meal	2 to 4 weeks	Stool character, post-fat bloating, retest popliteus. Fat-soluble vitamin level at 3 months if one is low.
Betaine HCl	With protein meals, mid-meal	2 to 3 weeks	Post-meal fullness, protein tolerance, reflux pattern. Warmth means reduce.
Digestive enzymes	First bite of the meal	2 to 4 weeks	Stool that no longer floats, less 30 to 60 minute bloating.
Probiotics	Consistently, timing matters less than consistency	4 to 8 weeks	Bristol type and frequency, bloating, urgency.
Fiber	With water, increase slowly	2 to 4 weeks	Stool form and frequency. Escalating too fast is the number one compliance failure.
Glutamine / GI repair	Away from meals	4 to 8 weeks	Food reactivity breadth, stool consistency, skin.
Adaptogens	Morning and early afternoon only	4 to 8 weeks	Sleep onset, 3pm energy, a validated stress scale.
Thyroid support	Away from calcium, iron and coffee	3 months	TSH, free T4, TPO antibodies. Labs, not symptoms.
Fat-soluble vitamins D, K, A, E	With a fat-containing meal	3 months	Serum 25-OH vitamin D. If it does not move, suspect absorption, see page 6.
Omega-3	With food, split if reflux occurs	3 months	Triglycerides. That is the biomarker with the strongest support.
Magnesium	Evening for most forms	4 weeks	Cramping, sleep, blood pressure. Loose stool means reduce or change form.
Berberine	Divided across meals	3 months	HbA1c, which reflects about 90 days.

The interactions worth memorizing right now

THE DRUG AND NUTRIENT INTERACTIONS WORTH MEMORIZING NOW

Knowing these cold is one of the fastest ways a young doctor earns respect from a medical colleague. Every one is mechanistic, not theoretical.

DEPLETIONS

- **Metformin depletes B12.** Systematic review and meta-analysis found a mean reduction of 57 pmol/L, enough to push some patients into frank deficiency. The authors recommend monitoring. *Chapman 2016.*
- **PPIs reduce B12, magnesium and iron absorption**, because all three depend on gastric acid.
- **Statins lower CoQ10** through mevalonate pathway inhibition.
- **Diuretics deplete potassium and magnesium** depending on class.

DO NOT COMBINE

- **Betaine HCl** with active ulcer, NSAIDs, PPIs or H2 blockers.
- **Vitamin K** with warfarin without physician coordination. This changes INR.
- **Iodine** in undiagnosed autoimmune thyroid disease.
- **Berberine** with drugs cleared by CYP450 or P-glycoprotein, and with glucose-lowering medication.
- **St. John's wort** with almost anything. It is a potent CYP3A4 inducer.

ABSORPTION COMPETITION

- **Calcium, iron and zinc** compete for shared transporters. Separate them.
- **Calcium and iron** block levothyroxine absorption. Four hours apart.
- **Fiber** binds medications. Separate by two hours.
- **Fat-soluble vitamins need fat** and, upstream of that, they need bile.

THE HABIT THAT OUTLASTS THIS HANDOUT

Take the medication list every time, including over the counter drugs and everything else the patient is already taking. Most supplement harm in the real world is not toxicity. It is an interaction nobody asked about, or a real diagnosis that got delayed while someone tried a bottle first.

You are going to get challenged on this.

Here is how you win that conversation.

Somewhere between now and your third year, someone is going to tell you applied kinesiology does not hold up. A professor, an MD at a case conference, a classmate, a patient's son with a search engine. If the first time you meet that literature is in the room, you lose. So meet it here instead.

WHAT HOLDS UP

- **Manual muscle testing as an assessment of muscle function.** It is used by MDs, DOs, DCs, physical therapists and athletic trainers, with a peer-reviewed history going back to 1915.
- **Specific muscles tested without challenge are reliable.** A 2025 systematic review in *JMPT* found piriformis at kappa 0.7 to 0.91, with deltoid, gluteus maximus and iliopsoas moderate to strong. *Soares 2025*.
- **AK as an adjunct, not a replacement.** The one independent study that came out weak-positive tested thyroid dysfunction and concluded AK "enhanced but did not replace" clinical and laboratory diagnosis. *Jacobs 1984*.
- **The nutrient biochemistry in this guide.** Every mechanism on pages 6 through 14 stands on its own literature, independent of how you found the patient.

WHAT HAS NOT HELD UP

- **Muscle testing plus a nutrient or allergen challenge.** This is the specific thing that has repeatedly failed under blinding.
- Eleven subjects, three experienced kinesiologists, four nutrients, against lab biochemistry and double-blind placebo: no interjudge reliability, no correlation with biochemical status, and no difference between placebo and nutrients. The authors wrote "no more useful than random guessing." *Kenney 1988*.
- Confirmed wasp venom allergy, fully blinded, 4 examiners: **kappa 0.03**. *Lüdtke 2001*.
- 315 children against RAST, IgG and lactose breath testing: test-retest kappa 0.62, **inter-tester kappa negative 0.01**. *Pothmann 2001*.
- 51 participants, sealed randomized vials, blinded: correct in 53 percent of trials, which is chance. *Schwartz 2014*.

READ THE PATTERN, BECAUSE IT IS THE INTERESTING PART

Practitioners are moderately consistent *with themselves* and essentially not consistent *with each other*. Kappa 0.62 within an examiner, kappa near zero between examiners. AK reads that as skill dependence. Critics read it as the ideomotor effect, meaning unconscious variation in the examiner's own applied force. **Both readings fit the data.** Saying that out loud, instead of picking a side you cannot defend, is what an honest clinician sounds like.

There is a real methodological complaint underneath it too. Force magnitude, duration and timing are almost never reported in AK studies, so nothing replicates cleanly and every negative result can be blamed on improper technique. *Conable & Rosner 2011*. That defense is unfalsifiable, and you should understand why that is a problem rather than reach for it.

How to win that conversation

THE MOVE THAT MAKES YOU UNBEATABLE IN THIS CONVERSATION

You never have to defend the muscle test as a standalone diagnostic, because you are never going to use it as one. That is not a retreat. It is **literally what your own governing body instructs**. The ICAK Status Statement says applied kinesiology examination "should enhance standard diagnosis, not replace it" and "is not intended to be used as a single method of diagnosis."

So run the Four Questions. The finding generates a hypothesis. History, symptoms, exam and labs confirm it or kill it. The recommendation rests on **biochemistry that is true regardless of how you found the patient**. Bile emulsifies fat whether you got there by popliteus, by a symptom survey, or by a GGT on a routine panel.

THE MUSCLE TEST IS THE DOORWAY. THE BIOCHEMISTRY IS THE LOAD-BEARING WALL.

Build the recommendation on the wall and it stands even if someone argues about the doorway. Build it on the doorway alone and one paper knocks the whole thing down.

TWO SCRIPTS. USE THEM VERBATIM UNTIL THEY ARE YOURS.

WHEN A COLLEAGUE SAYS AK HAS BEEN DEBUNKED

"The blinded studies on muscle testing as a standalone diagnostic are not favorable, and I know them. That is why I do not use it that way. I use it to generate a hypothesis, then I confirm it with history, exam and labs before I act. ICAK's own position statement says the same thing. What I recommended here is supported by the physiology and I am happy to walk you through it."

WHEN A PROFESSOR ASKS YOU TO JUSTIFY THE RECOMMENDATION

"The finding put the gallbladder on my differential. What kept it there was the fat intolerance, the stool description, and a vitamin D level that did not respond to eight months of supplementation. The recommendation is bile support, because micelle formation is required for fat-soluble vitamin absorption. I would expect her D to move at three months, and if it does not, I was wrong and I will say so."

WHEN A PATIENT ASKS WHETHER THIS IS REAL

"The test told me where to look. What convinced me was that three other things you told me pointed the same direction. Here is the actual reason I think this will help you, and here is what we should see change in about a month. If it does not change, that tells us something too and we will go a different direction."

WHY WE PRINTED THIS PAGE AT ALL

A supplement company handing you a document has an obvious incentive to skip this section. We put it in because the alternative is worse for you. If the first time you meet the Kenney paper or the Schwartz trial is when a resident quotes it back at you in front of a patient, we did not prepare you, we exposed you.

There is a version of applied kinesiology that cannot survive that conversation, and it is the version that treats the muscle test as proof. There is another version that survives it easily, because the test was only ever the thing that told you where to look, and the recommendation always rested on physiology you can cite. **The second version is the one worth building a career on.** Everything in this guide is written to put you there.

Check every one of these

Everything cited in this guide, with PubMed identifiers. We would rather you verify us than trust us. If you find something we got wrong, tell us and the next edition fixes it with your name on the correction.

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WHERE THE CHART ITSELF COMES FROM

Reflex point numbering follows the **Organ and Gland Receptors / Neurolymphatic Reflexes** chart, original source Robert A. Rakowski DC DIBAK, design and Swedish edition Mac Pompeus Wolantis, Svenska Kinesiologiskolan. Muscle, meridian and organ correspondences cross-checked against Walther's *Applied Kinesiology: Synopsis* (Systems DC) and the published AK correspondence tables. Product associations follow NutriDyn's applied kinesiology product reference, revision 1/22. Where sources disagree, this guide prints both and says so.

This is edition one.

You are the editors.

We built this from one conversation about one frustration. It is a draft on purpose. Tell us what is wrong with it, what is missing, and what you would actually use on the table, and edition two gets built around your answers.

WHAT WE WANT FROM YOU

CORRECT US

Three chart pairings in here are flagged because their lineage is uneven. If your seminar training says otherwise and you can source it, we will print your correction and credit the club.

TELL US WHAT IS MISSING

Which systems do you hit most in student clinic? Those get full worked examples like page 6 instead of a card.

TELL US THE FORMAT

Laminated table card? Pocket version? Something for the practical exam? We will build whichever one you would actually carry.

WHAT NUTRIDYN WILL PUT BEHIND THIS CLUB

Test kits, and someone who will actually teach you to use them

Kits into the club's hands, plus a working session on running them properly rather than a box with no instructions. Bring your questions. We would rather spend the hour on the four or five things that confuse everybody than deliver a lecture.

Product education that is real education, and support for your meetings

Formulation-level teaching. What is in it, why that form, what dose the research used, and where the evidence is thin. If a product has no mechanism behind it, we will tell you that too. A company that names its own weak spots is one you can trust on the strong ones. Plus food and materials for Thursday meetings, speaker support, and help getting AK practitioners in front of the club. If there is an ICAK meeting you want people at, let us talk early enough for it to matter.

The bridge to practice

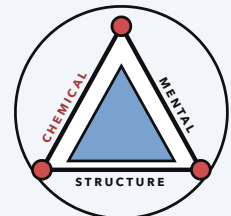
You graduate into a profession where nutrition is the fastest-growing part of a chiropractic practice and almost nobody was taught how to run it. Practice setup, protocol design, patient education, the business side of a dispensary. That help does not start at graduation. It starts now.

YOUR NUTRIDYN CONTACT

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On campus regularly, at Club Day every quarter, and reachable in between. If something in this guide is wrong, or you want the next edition to cover something specific, that is the email.



Scope and disclaimer. This guide is educational material prepared for chiropractic students and is not clinical advice, diagnosis, or a treatment protocol. Students must work within their program's scope and under supervision. Dietary supplements are not intended to diagnose, treat, cure or prevent any disease. Product-specific dosing must be taken from the current product label. Applied kinesiology procedures are, per the ICAK Status Statement, intended to enhance standard diagnosis rather than replace it. This document is prepared by NutriDyn for the Applied Kinesiology Club at Life University. It is not endorsed by, and does not represent the positions of, Life University or the International College of Applied Kinesiology.